Postpartum Hemorrhage

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Postpartum Hemorrhage

Introduction

- In Egypt, postpartum hemorrhage is the attributed cause for 32% of all maternal deaths, and 46% of all direct maternal death. ninety nine percent of all postpartum hemorrhage deaths were avoidable.

Definition

- Postpartum hemorrhage (PPH) is excessive blood loss at delivery affecting the general condition of the mother, a rising pulse rate, falling blood pressure and poor peripheral perfusion. Definition based on the amount of hemorrhage (blood loss of 500 ml or more from or within the reproductive tract after birth within 24 hours of delivery)

Types

- Primary postpartum hemorrhage occurs during the first 24 hrs after delivery.
- Secondary postpartum hemorrhage. Hemorrhage also may be delayed, occurring more than 24 hours after delivery. It can occur as long as 6 weeks after delivery.

Primary Postpartum Hemorrhage

Major Causes

Atonic Uterus:

- Atonic uterus is the commonest cause of postpartum hemorrhage with separation of the placenta, the uterine cannot be compressed effectively.

- Factors affecting efficient uterine contraction and retraction.
  - Placental
    - Incomplete separation of placenta.
    - Retained cotyledon, placental fragment or membranes.
    - Palcenta previa.
  - Prolonged labor
  - Multiple pregnancy or polyhydramnios.
  - General anesthetics.
  - A full bladder.
  - Manipulation of the uterus during third stage.

Traumatic:

- Hemorrhage occurs due to trauma of the uterus, cervix, vagina following spontaneous or operative delivery.
- Delay during episiotomy, laceration.

Mixed:

- Combination of atonic and traumatic causes.

Blood Coagulation Disorders:

- Acquired or congenital blood coagulation disorders are the factors sometimes causing postpartum hemorrhage.

Prevention

Antepartum

- Complete history should be taken to identify high-risk patients who are likely to develop PPH.
- Improvement of health status specially to raise the hemoglobin level.
- Hospital delivery of high-risk patients who are likely to develop PPH. e.g. polyhydramnios, multiple pregnancy, grand multipara, APH and severe anemia.
- Routine blood grouping and typing for immediate management during emergency.

Intrapartum

- Careful administration of sedatives and analgesic drugs.
- Avoid hasty delivery of the infant.
- Prophylactic administration of oxytocic drugs with delivery of anterior shoulder or at the end of third stage.
- Avoid massaging the uterus before separation of the placenta.
- Examine the placenta and membranes for completeness.
- Examine the utero-vaginal canal for trauma and prompt repair if present.
- Effective management of the fourth stage.

Control Bleeding by Using the Following Steps

- Exploration of uterus under general anesthetic.
- Bimanual compression (Uterus is firmly squeezed between 2 hands)
- Tight intrauterine packing to exert direct hemostatic pressure to stimulate uterine contractions.
- If all the above measures fail to achieve hemostasis a hysterectomy is performed.
- In traumatic PPH. speculum examination to find out trauma and hemostasis is achieved by appropriate sutures.

Observation of the Mother

- Record pulse and BP every 15 minutes.
- Palpate uterus every 15 minutes to ensure that it is well contracted.
- Cheek temperature 4 hourly.
- Examine lochia for amount and consistency
- Examine IV infusion.
- Hourly urine output.
- Intake and output chart.
- Relieve anxiety by explaining her condition and management.
- Administer prophylactic antibiotics prescribed considering the risk for infection.

Nursing Management of Postpartum Hemorrhage

Assessment

- **Identify Risk Factors in the Patient's History**
- Assess:
- Vital signs and general condition.
- State of uterus.
- Nature of bleeding.
- Signs and symptoms of blood loss.
- Amount of blood loss.
- Compare laboratory reports.

Nursing Interventions

- If atonic uterus:
  - Inform the obstetrician. Feel consistency of the uterus.
  - Massage the uterus to express clots and make it hard as follows. The fundus is first gently felt with the fingertips to assess its consistency. If it is soft and relaxed the fundus is massaged with a smooth circular motion, applying no undue pressure. When a contraction occurs the hand is held still.
  - Assess the general physical condition of the mother. (face, skin...)
  - Monitor TPR and blood pressure.

- Put the infant to the breast to suck or stimulate the nipple manually.
- Prepare instruments and equipment such as sterile gloves, cannula # 18, IV set, catheter set.... etc.
- Administer oxytocics as ordered.
- Start IV infusion and oxytocin drip.
- Empty the bladder.

– Examine the expelled placenta and membranes for completeness.

– Administer medications as ordered.

- Reassure the mother:

  - Never leave the mother alone.
  - Touch the mother’s hand and talk to her.

- In cases of traumatic bleeding:
  - Press on the tear or laceration.
  - Prepare equipment and instruments, sterile gloves, sterile needles and catgut, sterile needle holder, forceps, sterile kidney basin, scissors, sterile gauze etc.

Secondary Postpartum Hemorrhage

- Commonly occurs between 10 to 14 days after delivery.
- Common causes:
  - Retained bits of cotyledon or membranes.
  - Separation of a slough exposing a bleeding vessel.
  - Subinvolution at the placental site due to infection.

Clinical Manifestations

- Sudden episodes of bleeding with bright red blood of varying amounts.
- Subinvolution of uterus.
- Sepsis.
- Anemia.

Nursing Management

- Follow the same steps as in the case of postpartum hemorrhage due to retained parts of placenta.
- In cases of postpartum hemorrhage due to infection the following should be done:
  - Reassure the mother.
  - Monitor TPR and blood pressure.

- Start IV infusion and blood transfusion according to doctor’s orders.
- Prepare sterile instruments and equipment needed for examination.
- Empty the bladder.
- Administer medications as ordered (broad spectrum antibiotic).
- Follow strict aseptic technique while providing care to the woman.
- Frequent changing of sanitary pads.